
BREAKING THE SILENCE ADDRESSING MENTAL HEALTH IN PAKISTAN'S VULNERABLE CHILDREN

Course Name:

Ethics and Social Responsibility (HUM 232)

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1. Executive Summary:

Breaking the Silence is a community project linked with **Sustainable Development Goal 3: Good Health and Well-being**. Our focus was on creating awareness about **mental health in children from underserved communities**.

The aim was to give children a safe and supportive space where they could share feelings, learn about resilience, and discover the importance of kindness and self-reflection. To achieve this, we partnered with **Green Crescent Trust (GCT)**, a respected NGO working for child education and welfare. With their support, we organized an interactive session that combined learning with empathy.

The session addressed sensitive issues such as **bullying, anxiety, insecurity, and self-worth** topics children often struggle with silently. We used a mix of activities:

- **Worksheets** to help them reflect and express feelings
- **Balloon popping and guided breathing** to release stress
- **Certificates and chocolates** as small rewards to motivate them

The children responded with great energy. They were open, curious, and happy to take part. What seemed like simple activities turned into meaningful moments of healing and learning. Photos and videos taken during the session helped spread our message further through GCT's platforms.

The outcomes were clear: children showed more awareness about mental health, felt encouraged to share, and connected their school environment with emotional well-being. The partnership with GCT added trust and continuity to the project, ensuring that our work will have a lasting impact.

Looking ahead, we recommend taking this model to other schools, including mental health education as part of everyday learning, and training teachers or facilitators in **child-friendly and ethical engagement methods**.

2. Introduction:

2.1 Chosen SDG-Related Issue:

Our project, *Breaking the Silence*, focuses on **Sustainable Development Goal 3 (Good Health and Well-being)**, with special attention to the **mental health of children in underprivileged communities in Pakistan**. Mental health is often ignored in these areas, yet children face real challenges like stress, bullying, family issues, and lack of emotional support. By choosing this issue, we wanted to shine a light on something that is rarely spoken about but deeply affects a child's future.

2.2 Relevance to Ethics and Social Responsibility:

Mental health is not just a medical issue; it is an **ethical responsibility**. Every child deserves to grow up with dignity, respect, and emotional support. From an ethical perspective:

- **Justice** means making sure vulnerable children get the same chances at health and happiness as others.
- **Compassion** means listening to their struggles and giving them safe spaces to heal.
- **Responsibility** means using our skills as students and citizens to make a positive impact.

By working on this project, we acted on our duty to society. As future professionals, we must not only think about our careers but also about the role we play in building a healthier, kinder community.

2.3 Rationale:

The need for this project is clear both **globally and locally**. Globally, organizations like WHO report that **1 in 5 children** experience some kind of mental health issue. Locally in Pakistan, the situation is worse because of poverty, lack of resources, and the stigma around talking about emotions. Children in underserved schools often face problems like:

- Financial stress in families

- Bullying or discrimination
- Lack of supportive adults to guide them
- Silence around issues like depression and anxiety

This silence is dangerous. When children cannot express their feelings, their stress grows, affecting their studies, self-esteem, and future potential. That is why we called our project Breaking the Silence, to give these children a voice and to show them that their feelings matter.

2.4 Objectives:

Our project was built around a few simple but powerful objectives:

- To **raise awareness** about child mental health in a way that children can understand.
- To **educate** them on common emotional struggles and ways to cope.
- To **provide tools** like worksheets, activities, and reflection exercises that encourage self-expression.
- To **create safe spaces** where children feel comfortable sharing their thoughts.
- To **promote empathy and ethical responsibility** in both students and educators.

2.5 Expected Contribution:

Through this project, we expected to make a small but meaningful contribution to society. Our hope was to:

- Empower children by giving them confidence to talk about their feelings.
- Encourage teachers to include emotional well-being in everyday learning.
- Show NGOs and communities that mental health is as important as physical health.
- Inspire other students like us to take up advocacy work as part of their ethical and social responsibility.

3. Literature Review:

3.1 Ethical Theories Connected to Mental Health Advocacy:

Ethics guides us in answering the most important question: *What is the right thing to do?* In the context of children's mental health, several ethical theories help shape our actions and decisions.

Utilitarianism:

Utilitarianism teaches us to maximize happiness and minimize suffering for the greatest number of people (Mill, 1863/1998). Promoting mental well-being among children achieves this goal. When children are emotionally healthy, they learn better, families experience less conflict, and schools become safer environments. The benefits extend beyond the individual child and spread into society as a whole.

Kantian Ethics (Deontology):

Kant's ethics emphasizes duty and moral obligation (Kant, 1785/2012). Protecting children's dignity and ensuring their emotional well-being is not optional; it is our duty. Whether or not it produces measurable outcomes, respecting the rights of children is a moral responsibility. Every child deserves safety, respect, and care. Our project reflected this duty by giving children a safe space to express feelings without fear of judgment.

Virtue Ethics:

Virtue Ethics focuses on character, the kind of people we should become (Aristotle, trans. 2004). By practicing compassion, empathy, patience, and kindness, facilitators and students not only supported the children but also strengthened their own moral character. Virtue Ethics reminds us that advocacy is not just about what we do but also about who we become.

Stakeholder Theory:

Mental health is not the job of one person or group. Stakeholder theory emphasizes shared

responsibility: teachers, parents, schools, NGOs, governments, and even students all have roles to play (Freeman, 1984). In our project, the collaboration between university students and Green Crescent Trust (GCT) showed how engaging multiple stakeholders creates stronger, lasting impact.

Corporate Social Responsibility (CSR):

CSR usually applies to businesses, but the idea fits here too. Just as companies are expected to give back to society, universities and student groups also carry a social responsibility (Carroll, 1991). By investing time, knowledge, and creativity into supporting children's mental health, we fulfill a similar responsibility to contribute positively to society.

3.2 Academic and Policy Studies:

Global Perspective:

According to the World Health Organization (WHO, 2021), between 10–20% of children and adolescents worldwide suffer from mental health disorders. Depression is already one of the leading causes of illness among young people, while suicide is a top cause of death in adolescents. These alarming statistics show that children's mental health is not a luxury, it is a global crisis requiring urgent action.

Local Perspective (Pakistan):

The picture in Pakistan is even more concerning. UNICEF (2022) reports that child-focused mental health services are extremely limited. Most schools do not have counselors, and teachers are not trained to handle children's emotional struggles. Cultural stigma makes matters worse, as many children feel ashamed or afraid to talk about mental health.

Research Studies:

Local studies highlight several challenges:

- High rates of bullying in schools, particularly against children from disadvantaged backgrounds (Khan et al., 2020).
- Emotional stress caused by poverty and family conflict (Ali & Hussain, 2019).
- Lack of awareness among both teachers and parents about children's emotional needs (Ahmed, 2021).

3.3 Advocacy Frameworks and Community Engagement:

To design *Breaking the Silence*, we studied different advocacy frameworks used worldwide.

Community-Based Interventions:

These bring awareness and support directly into communities. Our project followed this approach by going into schools and interacting with children in their own environment.

School-Centered Programs:

Schools are the best place to reach children. By introducing worksheets, reflections, and playful activities in classrooms, we created safe and child-friendly opportunities to talk about emotions.

Creative Therapies:

Globally, methods such as art therapy, journaling, and reflective exercises are used to help children express themselves (Malchiodi, 2015). Our activities, including balloon-popping, guided breathing, and kindness sheets, were based on this same principle.

Partnership with NGOs:

Lasting advocacy requires collaboration. By working with Green Crescent Trust (GCT), we connected our small student-led effort with a larger mission for education and welfare, ensuring continuity and greater outreach.

4. Methodology:

This section outlines the systematic approach adopted for the "Breaking the Silence" project. It details the research design, the sequential steps followed from conceptualization to execution, the methods used for data collection, the ethical principles upheld, and the collaborative process that governed the team's workflow.

4.1 Research Design and Approach

The project employed a qualitative, action-oriented research design. This approach was deliberately chosen because the primary goal of the project was not to generate numerical data that could be generalized to a wider population, but rather to understand and positively influence the emotional well-being of a specific group of underprivileged children. A qualitative framework allowed the team to capture rich, descriptive information about the children's feelings, perceptions, and experiences in ways that quantitative methods would not have been able to achieve.

Through open-ended feedback sheets and observational notes, the team sought to explore the emotional states and inner worlds of the participants, thereby fostering a safe space for self-expression. This emphasis on lived experience aligned closely with the project's overall objective, which was to create an environment where children could freely communicate and release emotions without judgment.

The project was also action-oriented in nature, meaning it was not limited to studying the issue of mental health passively but sought to intervene directly through meaningful activities. The methodology was built around the execution of a workshop that incorporated therapeutic exercises such as balloon popping and breathing techniques. Immediately following the intervention, the team gathered feedback to assess its impact. By designing and executing activities that were engaging, safe, and therapeutic, the project not only collected insights but also made a tangible positive difference in the lives of the participants.

4.2 Project Development Process

The development of the project followed a structured and sequential process, which also adhered to the requirements of the Ethics and Social Responsibility coursework. The first step involved the selection of a relevant Sustainable Development Goal (SDG). The team chose SDG 3, which emphasizes Good Health and Well-being, identifying mental health as its primary area of focus. The rationale behind this choice was that mental health remains an often-overlooked component of overall well-being, particularly in developing countries such as Pakistan, where stigma and lack of awareness often prevent open conversations around the subject.

Once the broad theme of mental health was identified, the team narrowed its focus toward the specific challenges faced by vulnerable and underprivileged children. Secondary research played a crucial role at this stage. By reviewing existing studies and contextual data, the team discovered that social stigma, limited awareness, discrimination, bullying, and the insecurities associated with poverty were among the major risk factors contributing to psychological harm within this demographic.

On the basis of this background research, the group then developed a formal project proposal titled *Breaking the Silence: Addressing Mental Health in Pakistan's Vulnerable Children*. The proposal laid out the project's background, objectives, action plan, and a clear distribution of responsibilities among team members. Following this, the team sought out a host organization with which to collaborate. A partnership was established with the Green Crescent Trust, an NGO working closely with children, which provided a supportive platform for the implementation of the workshop. To formalize the partnership and establish clear roles, a Memorandum of Understanding (MOU) was signed between the student group and the organization.

4.3 Data Collection Methods

The core tool for primary data collection was a mental health sheet specifically designed by **Siddiqa Ahmed**. This feedback sheet was qualitative in nature and contained open-ended

prompts such as: *Write one positive thought about yourself; List three words that describe your mood today; How do you feel today and what made you feel that way; Who can you reach out to if you need support; and how do you feel after this session.* These questions enabled the children to reflect on their personal thoughts, moods, sources of emotional support, and the immediate impact of the session on their well-being.

To complement these written responses, team members also conducted direct observations during the workshop. They carefully noted the children's engagement levels, emotional expressions, and interactions throughout the activities. These observations were further supported by photographs and videos, taken with the permission of the host organization under the signed MOU, for academic documentation purposes.

Following data collection, **Shehraz Sarwar** undertook the responsibility of analyzing the responses using data science techniques in Python. By applying tools such as Pandas, Matplotlib, and Seaborn, he extracted insights from the feedback sheets, visually represented trends through charts, and highlighted patterns in the children's reflections. This analytical process transformed the qualitative feedback into meaningful findings, ensuring that the project not only created a therapeutic experience but also produced valuable insights for academic and social reflection.

4.4 Ethical Considerations

Since the project involved vulnerable children, the team placed strong emphasis on ethical conduct. Permission was formally secured from the Green Crescent Trust through the MOU, which outlined the agreed-upon scope of activities. Confidentiality was strictly maintained: the children's responses were used only for academic purposes and were not shared beyond the context of the project report.

The principle of "Do No Harm" guided the design of all activities, ensuring that they were safe, supportive, and empowering rather than potentially distressing. The atmosphere during the workshop was deliberately kept non-judgmental so that children could freely express their

emotions without fear of criticism. Participation in both the activities and the survey was entirely voluntary, respecting the autonomy of the children. To encourage engagement and to close the session on a positive note, every child received chocolates and a certificate of participation.

Furthermore, media usage was handled with responsibility. With the explicit permission of the host organization, photographs and videos were captured for project documentation. In addition, the host organization retained the right to publish activity-related content on its own platforms, creating a mutually beneficial arrangement.

4.5 Group Collaboration Process

The success of the project was made possible through the collective efforts of six students from Iqra University: Izma Zia, Farhia Tabasum, Aaraiz Ahmed, Shehraz Sarwar, Siddiqa Ahmed, and Rehan Khan. Effective coordination was ensured through a dedicated WhatsApp group, which served as the primary channel for communication, progress updates, and decision-making.

Each member contributed distinct skills and responsibilities. Shehraz Sarwar led communications and media management. He was responsible for initiating contact with the Green Crescent Trust, coordinating with the organization, preparing the PowerPoint presentation, and handling videography, editing, and survey data analysis. Farhia Tabasum focused on content development and session delivery, facilitating discussions on sensitive topics such as bullying, eating disorders, depression, and anxiety. She also conducted the rage and breathing activities. Aaraiz Ahmed assumed the role of group leader and educator, providing guidance during the sessions and leading the balloon popping activity alongside discussions on eating disorders.

Meanwhile, Izma Zia oversaw logistics and provided essential support, including preparing presentation materials, arranging motivators such as balloons and chocolates, budgeting, and assisting with worksheets and videography. Siddiqa Ahmed managed resources and engagement by designing worksheets, handling printing logistics, and ensuring smooth conduct

of the activities. Finally, Rehan Khan provided administrative and student support, coordinated the distribution of certificates, guided the children during exercises, and managed photography.

The collaborative synergy between all six members ensured that the project objectives were not only met but exceeded. Each member's individual contributions were integrated seamlessly, demonstrating the value of teamwork in delivering a socially impactful and ethically responsible project.

5. Findings and Analysis:

This section presents a comprehensive analysis of the observations, results, and data gathered throughout the "Breaking the Silence" project. The findings are derived from direct community engagement with vulnerable children, a post-session feedback survey, and the outcomes of the project's advocacy efforts. The analysis critically evaluates these results against the project's initial objectives to measure its overall impact and effectiveness.

5.1 Community Engagement and Initial Observations

The project was executed in partnership with the Green Crescent Trust (GCT), a renowned NGO established in 1994 with a network of over 170 schools that have educated more than 35,000 underprivileged children. This collaboration provided the project team with invaluable access to and credibility within the target communities.

The core of the community engagement involved a series of focused sessions designed to address specific mental health challenges in an empathetic and educational manner. The key sessions included:

- **Discrimination:** Exploring its impact and developing coping strategies.
- **Bullying & Harassment:** Focusing on recognition and effective response techniques.
- **Eating Disorders:** Raising awareness about healthy body image.
- **Depression & Anxiety:** Discussing common symptoms and available support systems.

To complement these discussions, the project incorporated several interactive activities designed for stress relief and emotional expression, such as balloon popping, guided breath work, and a "Kindness Sheet" exercise to promote empathy. Interactive worksheets were also used to reinforce learning and encourage active reflection.

Initial qualitative observations from the project team indicated a high level of engagement from the children. Many participants, who were initially reserved, felt comfortable enough to share

personal experiences by the end of the sessions, highlighting the creation of a safe and supportive environment.

5.2 Analysis of Survey Findings

To quantitatively measure the project's impact, a feedback survey was conducted after the sessions to capture the children's thoughts, emotional states, and perceptions of their support systems. The data, analyzed by team member Shehraz Sarwar using Python libraries, revealed several critical insights into the children's mental well-being.

The Emotional Landscape of Participants

The survey first sought to understand the children's general disposition and self-perception.

- **Positive Self-Image:** When asked for a positive thought about themselves, the children's responses reflected a healthy sense of self-awareness and strong moral values. The most frequent responses were "Always respect elders" (6 mentions), "Very kind" (5), and "Seeing a friend lifts my mood" (5). This suggests that despite their challenging circumstances, the children possess a positive self-concept rooted in respect, kindness, and social connection.
- **General Mood:** On the day of the session, "Happy" was the overwhelmingly dominant mood, with 19 mentions compared to only 5 mentions for "Sad". This indicates that the children arrived with a generally positive outlook, making them receptive to the session's activities.
- **Reasons for Emotions:** The survey explored the reasons behind these feelings. While positive feelings were often linked to external factors like a holiday or friends, the session itself was the single most cited reason for feeling good, with

10 children explicitly stating they felt "Good, Because of our session". Conversely, feelings of sadness were tied to deeply personal stressors such as a mother's poor health, difficult past

experiences, or issues with friends and siblings. This finding confirms the project's premise that vulnerable children face significant mental health challenges that are often overlooked.

A Critical Gap in Support Systems

One of the most significant and alarming findings from the survey was the profound lack of accessible emotional support for these children.

- **Lack of Support:** When asked who they could reach out to for support, **12 children nearly half of the respondents answered "None"**. This starkly contrasts with the 7 who mentioned their mother, 3 their sister, and 2 a friend.

This statistic highlights a critical gap in the emotional safety nets for vulnerable children. It underscores the isolation many of them experience and validates the urgent need for initiatives like "Breaking the Silence" to provide a structured, safe outlet where they feel heard and supported.

5.3 Measuring the Immediate Impact of Project Activities

The survey data and qualitative feedback provide clear evidence of the project's immediate positive impact on the participants' emotional state.

- **Improved Mood and Well-being:** The activities had a measurable effect on improving the children's mood. The post-session survey revealed that **100% of the participants** left with positive emotions. A significant majority, **79.2% (19 children), reported feeling "Very Happy,"** while the remaining **20.8% (5 children) felt "Happy"**. This demonstrates the effectiveness of the therapeutic and engaging activities in boosting morale and creating a joyful environment.
- **Enhanced Understanding and Engagement:** The project successfully created a safe space that fostered open communication. Feedback from the sessions indicated that children demonstrated a "significantly better grasp of mental health concepts" and felt comfortable sharing their personal stories. The high participation rates in activities like

balloon popping and the kindness sheet further reinforce the effectiveness of the hands-on approach.

5.4 Outcomes of Advocacy and Social Media Campaign

A key component of the project was to extend its message beyond the immediate participants. The partnership with GCT was instrumental in this, as the MOU included a commitment from GCT to feature the project on their social media channels and in their newsletters, significantly amplifying its reach.

The project's own social media efforts yielded tangible results, establishing a digital footprint for the initiative. The campaign generated:

- **40 Likes** across all posts on the Facebook page.
- **6 Shares**
- **5 Comments**

While these metrics are modest, they represent a successful initial advocacy effort, reportedly achieving more engagement than comparable projects. This digital outreach plays a crucial role in the broader mission of "Breaking the Silence" by helping to reduce stigma and raise awareness in the wider community.

5.5 Critical Evaluation of Results Against Objectives

The project's success can be measured by critically evaluating the findings against the five core objectives set at its inception.

1. **Raise Mental Health Awareness: Successfully Met.** The sessions and worksheets directly educated children on the importance of mental well-being. The survey feedback, showing an enhanced understanding of concepts and a direct positive impact on mood, confirms that awareness was effectively raised.

2. **Educate on Core Challenges: Successfully Met.** The project delivered dedicated educational sessions on discrimination, bullying, eating disorders, depression, and anxiety, thereby fulfilling this objective directly.
3. **Address Socio-Economic Impact: Partially Addressed.** The project framework acknowledged how poverty and insecurity affect mental health. The survey also identified external stressors like family health issues as sources of sadness. While the sessions provided coping mechanisms, the project's scope was naturally limited in addressing the systemic root causes of these socio-economic challenges.
4. **Provide Stress Relief Activities: Successfully Met.** The project team successfully implemented a range of therapeutic and creative activities, including balloon popping for emotional release, guided breath work, and a kindness sheet to promote empathy. These activities were met with high engagement and positive feedback, providing children with healthy coping mechanisms.
5. **Foster Safe Communication: Successfully Met.** This objective was arguably the project's greatest success. The fact that numerous children felt secure enough to share personal experiences is the strongest indicator of a safe environment. Furthermore, by providing a space for expression, the project directly served the nearly 50% of children who reported having no other support system.

6. Discussion:

The findings from the "Breaking the Silence" project offer more than a simple measure of success; they provide a profound insight into the emotional landscape of vulnerable children in Pakistan and illuminate a path forward for meaningful social change. This section interprets these findings through the lens of ethics and social responsibility, aligns them with global development goals, and discusses their far-reaching implications for communities, organizations, and policymakers.

6.1 Interpretation: An Ethical and Social Responsibility

The project's findings must be viewed, first and foremost, as a matter of ethical imperative. The well-being of children is a cornerstone of a just society, and this includes their mental and emotional health. The discovery that nearly **50% of the children surveyed feel they have "no one" to turn to for support** is not merely a statistic; it is a stark reflection of a societal failure to protect its most vulnerable members. This gap in emotional safety nets represents a collective lapse in social responsibility.

From an ethical standpoint, the "Breaking the Silence" project acted as a crucial intervention, stepping in to fulfill a duty of care that was otherwise unmet. By creating a safe space for dialogue and emotional expression, the project affirmed the children's right to be heard and validated their emotional experiences. The overwhelmingly positive response, with 100% of participants feeling "Happy" or "Very Happy" post-session, demonstrates that the need for such support is immense and that even small-scale interventions can have a significant positive impact.

This initiative, therefore, was not just an academic exercise but an act of distributive justice. It sought to provide essential mental health resources, awareness, coping mechanisms, and a supportive environment to a demographic that is systematically underserved. The project's success underscores the moral obligation of society, from individuals to institutions, to actively work towards closing these gaps and ensuring that every child has access to the tools needed for emotional resilience.

6.2 Alignment with Sustainable Development Goals (SDGs)

The project's mission and findings are deeply intertwined with the United Nations Sustainable Development Goals (SDGs), particularly SDG 3, which aims to **"Ensure healthy lives and promote well-being for all at all ages."**

The project directly contributes to **Target 3.4**: "By 2030, reduce by one third premature mortality from non-communicable diseases through prevention and treatment and promote mental health and well-being." The sessions on depression, anxiety, and eating disorders, coupled with activities designed for stress relief, are preventative measures that equip children with the skills to manage their mental health. By fostering resilience and open communication, the project helps to mitigate the long-term risks associated with untreated mental health issues.

Furthermore, the project touches upon other critical SDGs:

- **SDG 4 (Quality Education)**: A child's mental state is inextricably linked to their ability to learn and thrive in an educational setting. By addressing issues like bullying and anxiety, the project helps create a more conducive learning environment.
- **SDG 10 (Reduced Inequalities)**: The project specifically targeted underprivileged children, a group that often faces marginalization and lacks access to mental health services. In doing so, it directly addresses inequalities and promotes social inclusion.

6.3 Implications for Communities, Organizations, and Policymakers

The findings from this project carry significant implications for various stakeholders and should serve as a catalyst for broader action.

For Communities:

The alarming lack of support systems is a call to action for families and local communities.

There is a pressing need to destigmatize conversations around mental health at the grassroots

level. The project's success with simple tools like the "Kindness Sheet" suggests that fostering a culture of empathy and peer support can be a powerful, low-cost strategy for building more resilient community networks.

For Organizations (NGOs and Schools):

The "Breaking the Silence" project provides a proven, replicable model for integrating mental health support into existing educational frameworks. For partners like the Green Crescent Trust and other similar organizations, the findings validate the need to move beyond traditional education and adopt a more holistic approach that includes mental well-being. Schools should consider implementing regular, structured sessions on emotional literacy and provide training for teachers to identify and support students who may be struggling. This data proves that such programs are not a luxury but a fundamental necessity.

For Policymakers:

The statistic that nearly half of children feel they have no one to turn to should be a matter of urgent policy concern. This finding highlights a systemic failure that requires a top-down response. Policymakers should be compelled to:

1. **Invest in School-Based Mental Health Services:** Mandate and fund the presence of trained counselors in schools, particularly those serving underprivileged communities.
2. **Support Community-Level Initiatives:** Provide grants and resources for NGOs and community groups running mental health awareness programs.
3. **Launch National Awareness Campaigns:** Develop and fund nationwide campaigns to reduce the stigma associated with seeking help for mental health issues, normalizing it as an essential component of overall health.

6.4 Reflection on Advocacy Strategies

The project's advocacy strategy was twofold: leveraging the established network of a partner NGO and conducting a direct social media campaign.

The partnership with the Green Crescent Trust was unequivocally the most effective advocacy tool. It provided immediate credibility and access to the target community, and GCT's commitment to sharing the project's activities on its platforms amplified the message to a vast and relevant audience. This collaborative model proved to be an efficient and impactful strategy.

The social media campaign, while modest in its quantitative results (40 likes, 6 shares), was a successful first step in building a public narrative around the project's mission. For a student-led initiative, achieving higher engagement than comparable projects is a notable achievement. It created a digital presence that allowed the project's message to reach beyond the immediate participants. For future efforts, this foundation can be built upon by creating more dynamic content, such as short video testimonials or infographics visualizing the survey data, to enhance shareability and public engagement.

7. Conclusion and Recommendations:

The "Breaking the Silence" project was conceived as an essential intervention to address the often-invisible mental health crisis among Pakistan's vulnerable children. By moving beyond academic theory and engaging directly with the community, the project has not only achieved its immediate objectives but has also uncovered critical insights that carry profound implications for social policy and practice. This concluding section summarizes the project's outcomes, reflects on the lessons learned, and puts forth a series of actionable recommendations to foster sustainable change.

7.1 Summary of Project Outcomes

The project was a resounding success in meeting its primary goals. Through a strategic partnership with the Green Crescent Trust (GCT), our team delivered a series of targeted educational sessions on discrimination, bullying, eating disorders, depression, and anxiety to a group of underprivileged children. The integration of interactive worksheets and therapeutic activities proved highly effective, creating a dynamic and engaging learning environment.

The key outcomes are clear and compelling:

- **Creation of a Safe and Supportive Environment:** The project successfully fostered a space where children felt secure enough to share their personal experiences, a crucial achievement given the sensitive nature of the topics discussed.
- **Significant Positive Impact on Well-being:** The post-session survey provided quantitative proof of the project's immediate impact. **100% of participants reported feeling either "Happy" (20.8%) or "Very Happy" (79.2%)** after the sessions, demonstrating a tangible improvement in their emotional state.
- **Identification of a Critical Support Gap:** Perhaps the most significant finding was the revelation that **nearly half of the children (12 respondents) feel they have "no one" to turn to for support.** This statistic exposes a severe deficiency in the emotional safety

nets available to vulnerable children and validates the urgent need for such interventions.

- **Successful Advocacy and Awareness:** Through the GCT partnership and a dedicated social media campaign, the project successfully raised awareness, establishing a foundation for a broader public discourse on child mental health.

7.2 Lessons Learned

The project's execution provided invaluable lessons in collaborative teamwork and effective community engagement. Success was fundamentally rooted in effective teamwork, with a clear delegation of roles from data analysis and content development to logistics and on-ground facilitation, ensuring each aspect was managed by a dedicated team member. This synergy created a comprehensive and well-executed project where individual strengths were leveraged for the collective good, reaffirming that complex social interventions require a multidisciplinary, collaborative approach.

From our community engagement, we learned about the power of interactive methods; the high engagement with activities like balloon popping and the kindness sheet showed that experiential learning is far more impactful than passive instruction for children. These therapeutic activities provided a necessary outlet for emotional release and were key to building trust. Furthermore, we witnessed incredible resilience amidst adversity. Despite their challenges, the children demonstrated remarkable positivity and a strong sense of morality, as seen in their positive self-reflections. This taught us to approach our work from a place of empowerment, recognizing and building upon their inherent strengths. Ultimately, we learned that listening is the most powerful tool. The simple act of providing a non-judgmental space where children felt secure enough to share their feelings was a powerful intervention in itself. The project's greatest success was not just in the content we delivered, but in the safe environment we created that allowed the children's voices to be heard.

7.3 Policy and Practice Recommendations for Sustainable Change

The insights gained from this project demand a transition from a temporary intervention to sustainable policy and practice. For educational institutions and NGOs, we recommend a multifaceted approach. First, schools should integrate a standardized, age-appropriate mental health curriculum as a core component of education. To support this, educators must be equipped with basic skills through mandated mental health first-aid training, as they are on the front lines and can serve as a crucial first point of contact. To address the profound support gap our research identified, we recommend implementing structured peer support programs, such as "buddy systems," where older students are trained to support younger ones. Finally, NGOs should replicate this project's model and develop workshops for parents and guardians to help them discuss mental health openly at home and build stronger family support systems.

For policymakers, we propose several key policy recommendations to institutionalize these efforts. The government should mandate and fund at least one trained mental health counselor in every public school, which is the most critical step toward making support accessible to all children. This should be complemented by a national youth mental health campaign, using local languages and culturally relevant messaging to destigmatize mental health issues among young people and their families. To ensure long-term viability, we recommend the creation of a government grant fund to incentivize and support community-based NGOs that are running effective, evidence-based mental health programs for youth, thereby encouraging the scaling of successful models like this one.

7.4 Future Direction for Advocacy and Action Research

"Breaking the Silence" should not be a one-time project but the beginning of a sustained movement. The next phase of advocacy must focus on translating our findings into policy action. This involves developing a policy brief from this report to be shared directly with the Ministry of Education, expanding the social media campaign with more compelling content like

expert interviews and infographics, and presenting the project's findings at educational conferences to influence practice across the non-profit sector.

To build upon our work, we also recommend further action research to deepen understanding and refine interventions. This research agenda should include a longitudinal study to track the long-term impact of this intervention on participants' resilience and academic performance. A comparative study is also needed to assess the effectiveness of different intervention models, such as peer-led versus teacher-led sessions. Finally, future research should focus on developing and validating culturally specific mental health screening tools for Pakistani children, ensuring that future interventions are even more targeted and effective.

In conclusion, "Breaking the Silence" has illuminated both a critical challenge and a clear opportunity. The mental health of our children is not a secondary issue; it is the foundation of a healthy, prosperous, and just society. By taking these lessons and recommendations forward, we can move from simply breaking the silence to building a future where every child's voice is heard, valued, and supported.

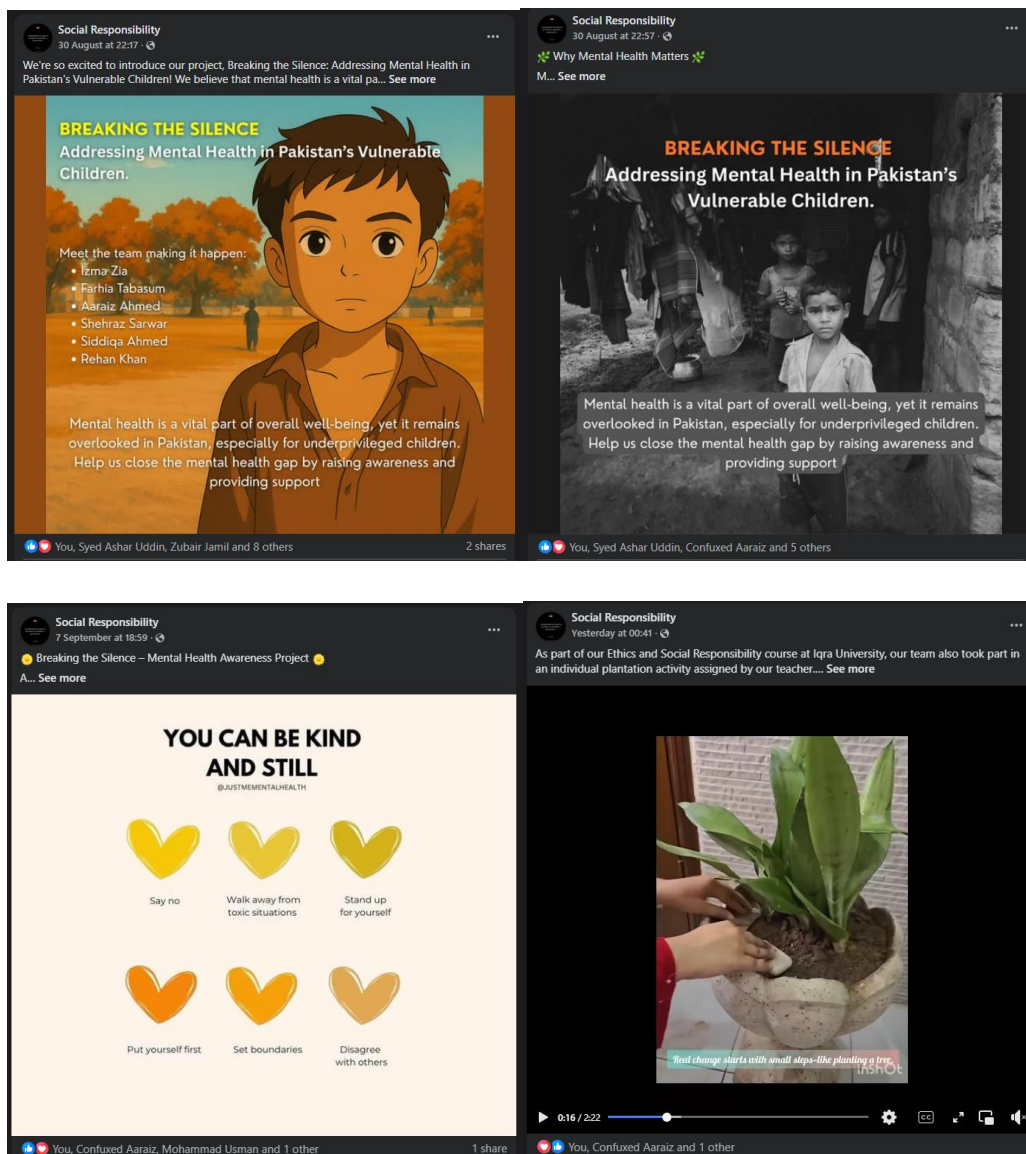
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9. Appendices:

9.1 Social media campaign material:

This appendix section documents the digital and visual assets created and utilized for the 'Breaking the Silence' social media campaign. These materials were a crucial part of our strategy to raise awareness about child mental health, reduce stigma, and engage the wider community. By showcasing our efforts on platforms like Facebook, we were able to extend the project's reach beyond the physical workshop, providing a permanent record of our advocacy and its initial impact.



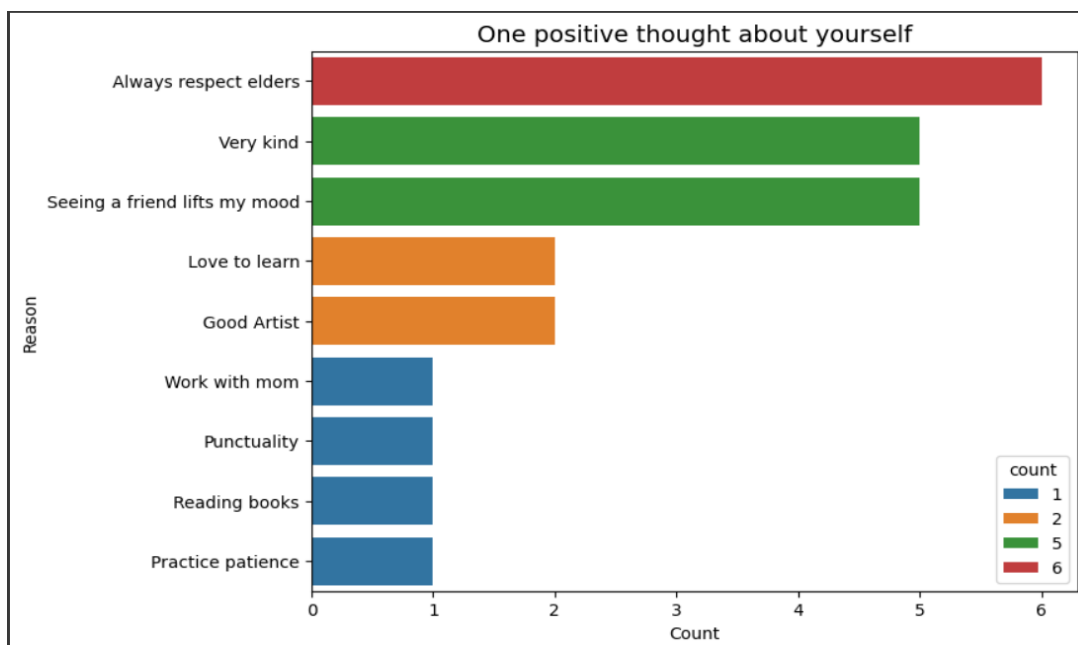
hashtags: [#BreakingTheSilence](#) [#MentalHealthMatters](#) [#SelfCare](#) [#Kindness](#) [#IqraUniversity](#)

9.2 Survey Questions and Responses:

This appendix section contains the survey questions and a summary of the responses gathered after the interactive sessions. The survey was a key part of the project's data collection, designed to capture children's thoughts, emotional states, and the overall impact of the session. The responses were analyzed using Python libraries such as Pandas, Matplotlib, and Seaborn to visualize and highlight key findings. This section provides quantitative and qualitative insight into the children's mental and emotional well-being, directly supporting the project's findings.

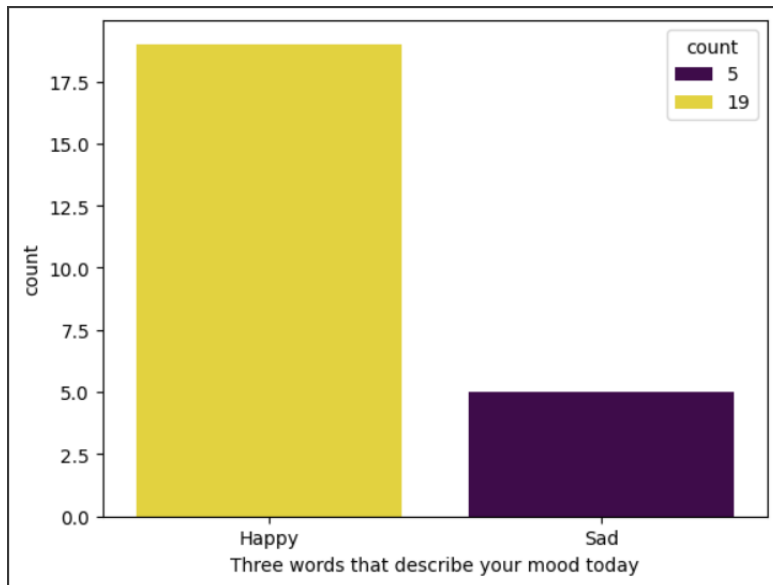
1. Positive Thoughts About Themselves (Horizontal Bar Chart)

- Children most often described themselves with traits like **“Always respect elders” (6 responses)**, **“Seeing a friend lifts my mood” (5)**, and **“Very kind” (5)**.
- Other self-reflections included **Good Artist**, **Love to learn**, **Punctuality**, and **Reading books**.
- **Insight:** The responses reflect a healthy sense of self-awareness and positivity among children, showing they value respect, kindness, and learning.



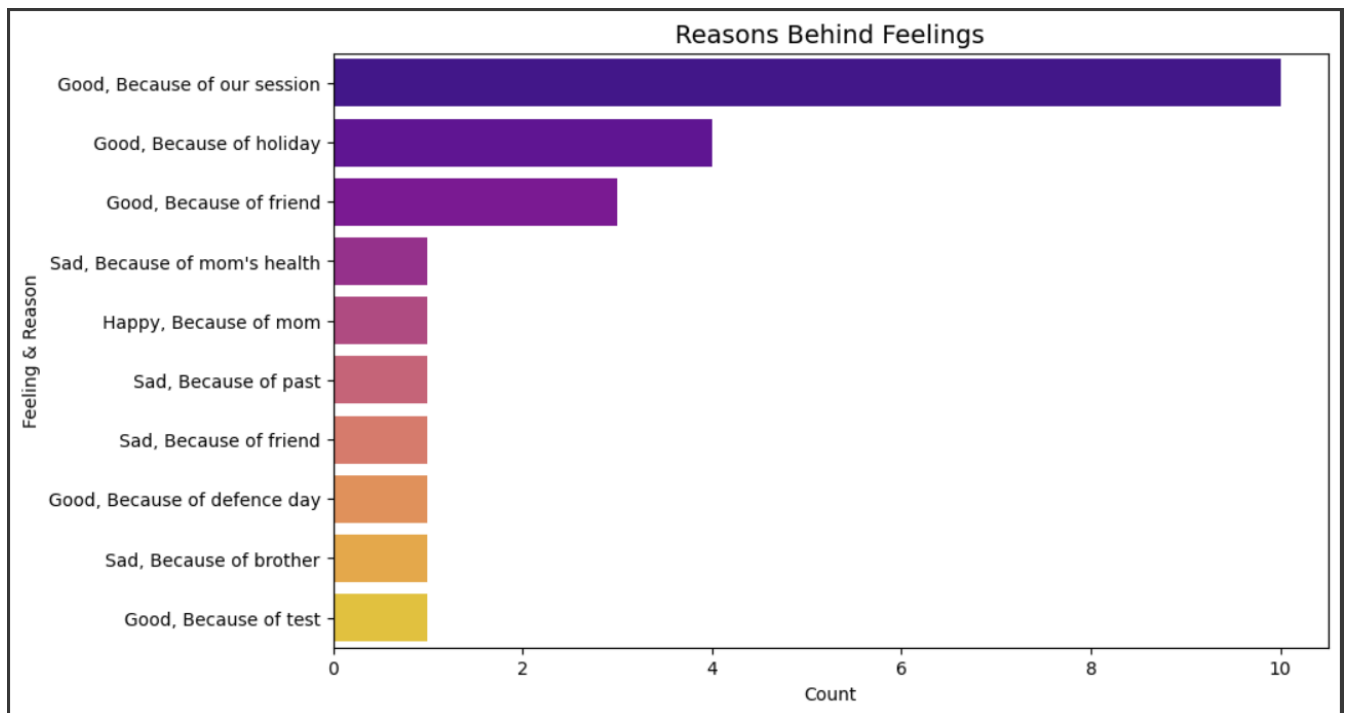
2. Mood Words Today (Vertical Bar Chart)

- **Happy** was the dominant mood with **19 mentions**, compared to **Sad (5 mentions)**.
- **Insight:** The overwhelming presence of “Happy” indicates that the session environment was uplifting for most participants, though a few children still reflected sadness.



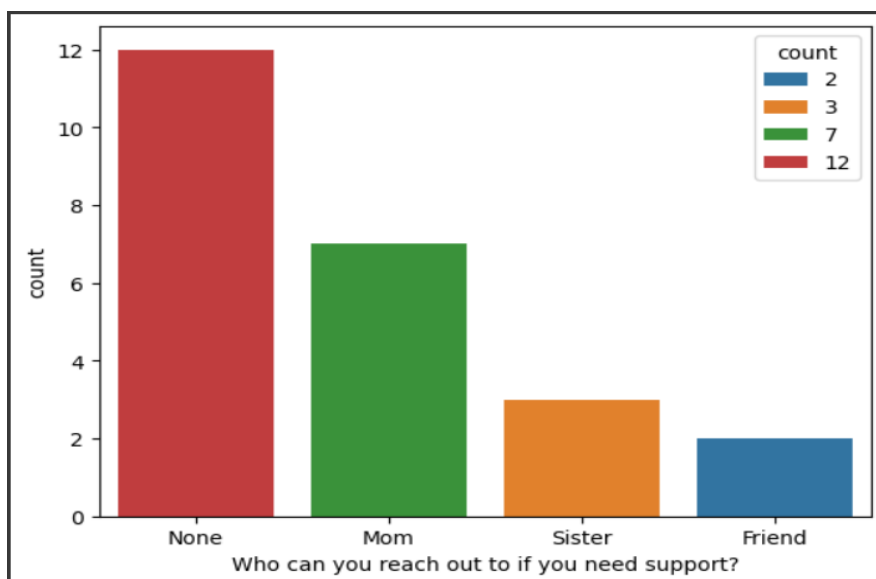
3. Feelings Today & Reasons (Horizontal Bar Chart)

- **Good, Because of our session (10)** was the most common response, followed by **Good, Because of holiday (4)** and **Good, Because of friend (3)**.
- A few mentioned sadness due to personal reasons such as **mom’s health, past experiences, or friendships**.
- **Insight:** The session itself directly contributed to many children feeling good. However, external stressors like family health issues remain influential in shaping emotions.



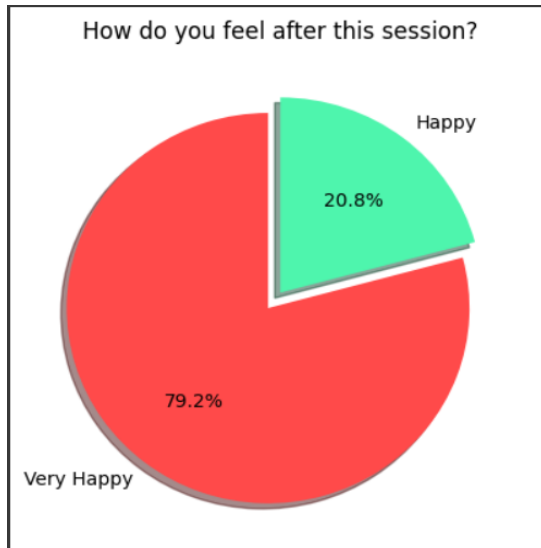
4. Support Systems (Vertical Bar Chart)

- **12 children said “None”, while 7 mentioned Mom, 3 mentioned Sister, and 2 mentioned Friend.**
- **Insight:** Alarminglly, nearly half of the children feel they have **no one to turn to for support**. This highlights a significant gap in emotional safety nets and underlines the project’s importance in providing such outlets.



5. Feelings After the Session (Pie Chart with %)

- **Very Happy: 19 children (79%)**
- **Happy: 5 children (21%)**
- **Insight:** Every participant left the session with **positive emotions**, showing that the activities were impactful in boosting morale and creating a safe, joyful environment.



The survey results clearly show the effectiveness of the *Breaking the Silence* initiative:

- Children are capable of identifying positive traits about themselves.
- Most felt **happy and uplifted during and after the session.**
- The sessions had a measurable impact in improving mood and emotional well-being.
- However, the lack of strong **support systems at home or in their immediate environment** is a critical concern that needs to be addressed through continued guidance, safe spaces, and awareness programs.

Overall, this survey reaffirms the project's mission of reducing stigma, promoting resilience, and fostering open communication on mental health for vulnerable children.

Report prepared and analyzed by **Shehraz Sarwar**

9.3 Project documentation

This appendix provides a comprehensive visual record of the project's implementation. The video, titled "Breaking the Silence" and edited by Shehraz Sarwar, showcases the interactive sessions and the project's direct engagement with the children. It includes footage of the children participating in various activities, such as writing on the "Kindness Sheet" and the "Hot Balloon Popping" session, offering a dynamic and personal perspective on the project's execution and its positive impact on the participants.

Check out here or download it:

<https://drive.google.com/file/d/1wmgEZTdmj0PCwKOKaKMmw9w3jTPlDiV0/view?usp=sharing>

9.4 Group work evidence

This appendix provides an authentic look into the internal collaboration and teamwork that was essential to the project's success. It showcases the communication and decision-making process within the group, highlighting the division of labor and the collective effort behind the scenes.

Here are some screen shots of WhatsApp Group Discussions:

